

Policy on Patient Feedback, Experience and Complaint Redressal

«Hospital Name»

Document control

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Prepared by	Quality Coordinator Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» has a mechanism to capture patient feedback and experience, and to redress complaints, so patient voices shape how care is delivered.

1. Purpose

This policy sets out how «Hospital Name» captures patient feedback and patient experience, redresses complaints, makes patients aware of how to give feedback or complain, reviews and analyses what it receives, and acts on what it finds.

This policy does not cover informed consent, patient education about healthcare needs, or information on costs — those are covered in other hospital policies.

2. Scope

This policy applies to feedback, experience data and complaints from all patients and families at «Hospital Name», across out-patient and in-patient settings, including complaints against healthcare workers.

3. Policy standards

«Hospital Name» has a mechanism to capture feedback from patients, including patient satisfaction, and a mechanism to capture the patient experience, beyond satisfaction, including patient-reported experience measures.

«Hospital Name» redresses patient complaints as per a defined, written mechanism; makes patients and families aware of the procedure for giving feedback and/or lodging complaints; reviews and/or analyses feedback and complaints within a defined time frame; and takes corrective and/or preventive action based on that analysis where appropriate.

4. Non-negotiable rules

- Do not operate without a mechanism to capture feedback from patients, including patient satisfaction.
- Do not limit patient experience capture to satisfaction alone — it must go beyond, and must include patient-reported experience measures.
- Do not redress patient complaints outside a defined, written mechanism — and do not exclude verbal, telephonic, or healthcare-worker complaints from that mechanism.
- Do not leave patients and families unaware of how to give feedback or lodge a complaint.
- Do not let feedback or complaints sit unreviewed beyond the defined time frame, or leave the review process undocumented.
- Do not leave a pattern or systemic gap identified through analysis without corrective or preventive action.
- Staff who see a rule under this policy broken report it the same shift to the Quality Coordinator or the Medical Superintendent.

5. What we do

5.1 Capturing patient feedback

A mechanism to capture feedback from patients — including patient satisfaction — exists at «Hospital Name», captured physically or electronically. Out-patient and in-patient data are kept separate as best practice.

5.2 Capturing patient experience

Beyond patient feedback, «Hospital Name» also captures patient experience — covering, at a minimum, communication with doctors and nurses, pain management, hospital environment (cleanliness and quietness), responsiveness of hospital staff, discharge information, communication about medications, and overall rating. «Hospital Name» captures patient-reported experience measures (PREMs) as part of this.

5.3 Complaint redressal mechanism

«Hospital Name» has written guidance covering the mechanism for lodging complaints (including verbal or telephonic complaints), the method of compiling and analysing complaints and the time frame for doing so, the person(s) responsible, and how action taken is documented. Patient complaints include those against healthcare workers. Whether to give credence to anonymous complaints is for «Hospital Name» to decide.

5.4 Awareness of the feedback and complaint procedure

Patients and families at «Hospital Name» are made aware of the procedure for giving feedback and/or lodging complaints, through display or written information. «Hospital Name» works to create an environment of trust in which patients are comfortable airing their views.

5.5 Review and analysis

Feedback and complaints at «Hospital Name» are reviewed and/or analysed within a defined time frame, and the entire process is documented. Where appropriate, the patient and/or family may be involved in the discussion and informed of the outcome.

5.6 Corrective and preventive action

Where the analysis in Section 5.5 identifies an opportunity for improvement, «Hospital Name» carries out corrective and/or preventive action.

6. Stop-work authority

Not applicable. This standard has no stop-work authority — gaps in feedback capture, complaint redressal or analysis are addressed through the hospital's quality monitoring and CAPA process (Section 8), not through an immediate stop-work trigger.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring the feedback, experience and complaint mechanisms are in place and effective; receives escalations under Section 4.
Quality Coordinator	Owns implementation of this policy; maintains the feedback and complaint mechanisms, complaint log and analysis records; tracks CAPA to closure.
Department Heads / Unit In-Charges	Support complaint investigation and implement corrective/preventive action in their area.
All Staff	Direct patients to the feedback and complaint channels; report identified risks and rule breaches.

8. Quality monitoring

Monitoring area	What is monitored
Patient feedback mechanism	Feedback tool or form; response rate record; tabulated satisfaction data with out-patient and in-patient data kept separate.

Patient experience capture	Experience capture tool showing the listed parameters; PREM/experience reports to the Medical Superintendent; action records.
Complaint redressal mechanism	Written complaint-mechanism guidance; complaint log (including verbal, telephonic, anonymous and healthcare-worker complaints); documented action records for sampled complaints.
Awareness	Display records (rights board, admission pamphlet, ward notice) matching the actual complaints desk/phone/form.
Review and analysis	Documented review/analysis process record; timelines for complaints and feedback; overdue-item list reported to the Medical Superintendent.
CAPA	CAPA records from feedback and complaint analysis, with owner, due date and closure.

9. Training and staff acknowledgement

All staff covered by this policy are trained on the feedback and complaint mechanisms described in Section 5 at induction and periodically.

I have read the Policy on Patient Feedback, Experience and Complaint Redressal of «Hospital Name» and understand my responsibilities under it.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to the Quality department, all department heads/unit in-charges, and all staff through the hospital intranet and department policy folders.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OPD	Out-Patient Department
PREM	Patient-Reported Experience Measure

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means the objective element is starred and requires documented evidence.

Objective Element	Level	Traceability to this policy
PRE.7.a	Commitment	Sections 3 and 5.1 address the mechanism to capture patient feedback, including patient satisfaction, with out-patient and in-patient data separate.
PRE.7.b	Achievement	Sections 3 and 5.2 address the mechanism to capture patient experience beyond satisfaction, including PREMs and the listed experience parameters.
PRE.7.c	CORE*	Sections 3 and 5.3 address the written complaint redressal mechanism, covering verbal/telephonic/healthcare-worker complaints and documented action.
PRE.7.d	Commitment	Sections 3 and 5.4 address making patients and families aware of the feedback and complaint procedure through display and written information.
PRE.7.e	Commitment	Sections 3 and 5.5 address review and/or analysis of feedback and complaints within a defined time frame, with the process fully documented.
PRE.7.f	Commitment	Sections 3 and 5.6 address corrective and/or preventive action based on feedback and complaint analysis where appropriate.

13. Required Records/Evidence Checklist

Patient feedback — PRE.7.a (Commitment)

- Feedback tool or form, response rate record against the target, and tabulated satisfaction data with out-patient and in-patient data kept separate.

Patient experience — PRE.7.b (Achievement)

- Experience capture tool showing the listed parameters (communication, pain, environment, responsiveness, discharge information, medications, overall rating).
- PREM/experience reports to the Medical Superintendent.
- Action records from those reports.

Complaint redressal mechanism — PRE.7.c (CORE*)

- Written complaint-mechanism guidance document covering lodging, compiling, analysing, time frame, responsible person, and documentation of action taken.
- Complaint log including verbal, telephonic, anonymous and healthcare-worker complaints.
- Documented action records for sampled complaints.

Awareness — PRE.7.d (Commitment)

- Display records (rights board, admission pamphlet, ward notice) naming the feedback and complaint procedure.
- Verification that the displayed path matches the actual complaints desk/phone/form.

Review and analysis — PRE.7.e (Commitment)

- Documented review/analysis process record and the defined review time frame.
- Overdue-complaint list reported to the Medical Superintendent.

CAPA — PRE.7.f (Commitment)

- CAPA records from feedback and complaint analysis, with owner, due date and closure.
- Confirmation that analysis led to action, not analysis alone.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PRE.7.

Disclaimer

This policy reorganises the supplied PRE.7 objective-element wording and Guidebook interpretation into plain-language guidance for «Hospital Name». It does not replace the NABH Accreditation Standards for Hospitals, 6th Edition, or the Guidebook, which remain the authoritative source in case of any conflict.

This policy is intended for accreditation and internal governance purposes and is not a statement of, or substitute for, compliance with any specific statute; «Hospital Name» remains separately responsible for meeting all applicable statutory and regulatory requirements.

This policy shall be reviewed at least once every year, or earlier if NABH standards change.

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